

Facility name

All

Sub region

All

Severity

All

144K

Total Discharges

24

Facility Count

14

County Count

2.27bn

Total Costs

2.95

Charge to Cost Ratio

FACILITY RISK SCORE — TOP 8 OF 24

Facility	Sub Region	Discharges	Facility Medicaid Share	Extreme Cost %	Score
ST. PETER'S HOSPITAL	Capital District	24768	20.85%	26.00%	57.41
SAMARITAN HOSPITAL	Capital District	9912	26.99%	26.32%	66.67
MARY IMOGENE BASSETT HOSPITAL	Central NY / Catskill Foothills	9910	16.17%	23.91%	53.70
ELLIS HOSPITAL	Capital District	9552	19.20%	33.21%	68.52
THE UNIVERSITY OF VERMONT HEALTH NETWORK - CHAMPLAIN VALLEY PHYSICIANS HOSPITAL	North Country / Adirondack	8072	18.64%	25.05%	61.11
ST. MARY'S HEALTHCARE	Mohawk Valley	3806	28.35%	15.34%	61.11
COLUMBIA MEMORIAL HOSPITAL	Central NY / Catskill Foothills	3051	15.37%	21.87%	57.41
NATHAN LITTAUER HOSPITAL	Mohawk Valley	2282	30.15%	17.00%	68.52
Total		71353	21.04%	25.90%	57.41

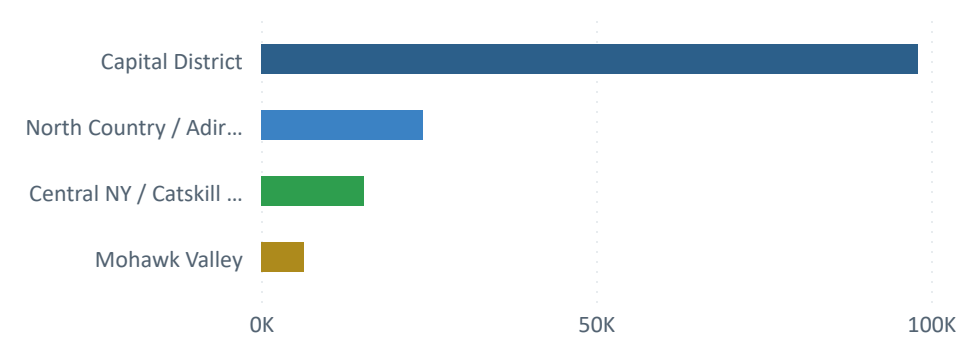
RECOMMENDED FOCUS AREAS

Ellis Hospital and Nathan Littauer Hospital tie for the highest Facility Risk Score (68.5) among the 19 general-acute facilities, driven by Ellis's high Extreme-tier cost share (33.2%) and Nathan Littauer's high Medicaid dependency (30.1%) at real volume.

Samaritan Hospital ranks third (66.7), combining above-average Medicaid share (27.0%) and Extreme-tier cost share (26.3%) at high volume (9,912 discharges) - a genuine scale risk, not a small-unit artifact.

Unlike the prior all-facility ranking, none of the current top 8 general-acute facilities are flagged low-volume, so this list reflects network-adequacy risk at hospitals with substantial patient volume, not specialty/satellite units of larger systems.

TOTAL DISCHARGES BY SUB REGION



Facility name

All

10.88%

Top 1% Discharge Cost Sh...

27.53%

Top 5% Discharge Cost S...

40.03%

Top 10% Discharge Cost S...

24.68%

Extreme Tier Cost Share

2.27bn

Total Costs

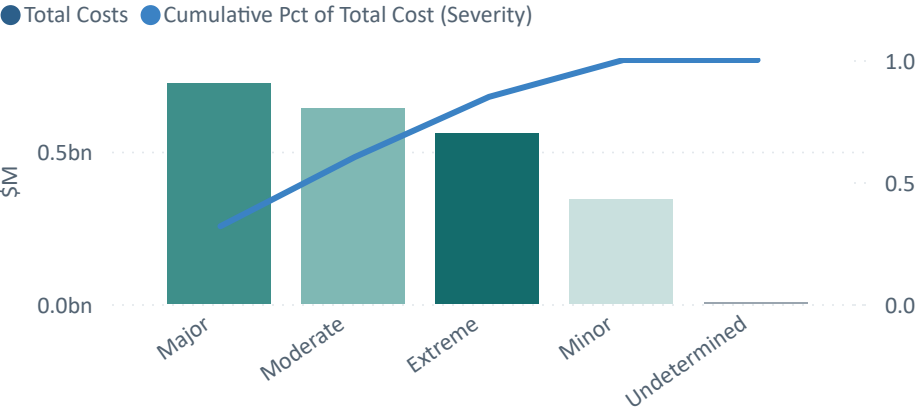
Sub region

All

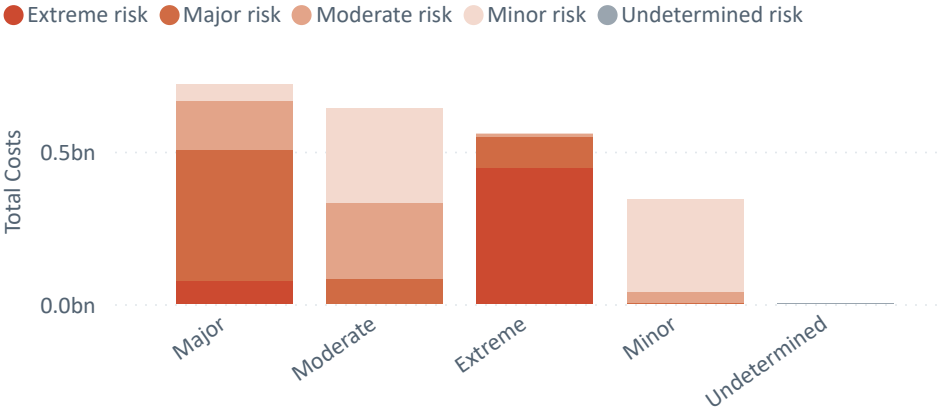
Severity

All

COST CONCENTRATION BY SEVERITY TIER (PARETO)



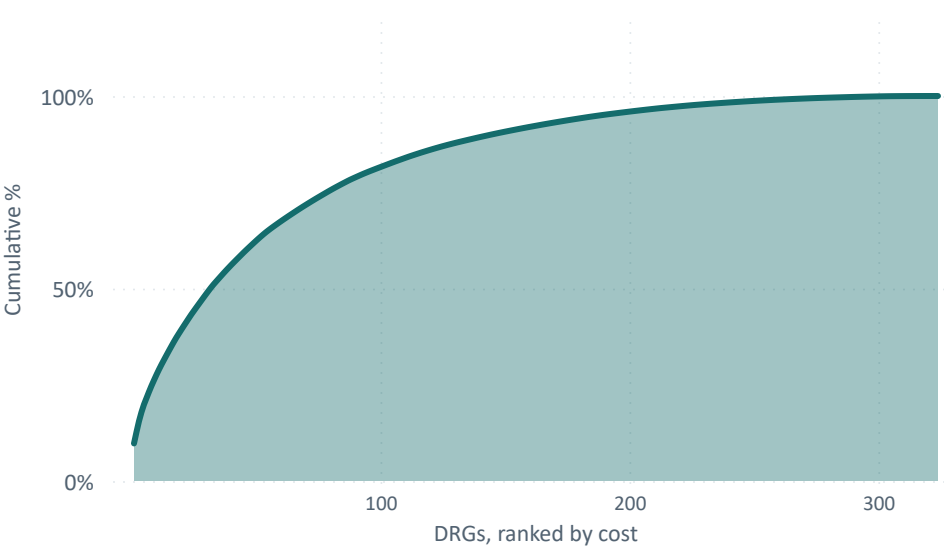
TOTAL COSTS BY SEVERITY AND RISK OF MORTALITY



TOP 15 DRGS BY TOTAL COST (TOP 7 SHOWN)

DRG description	Discharges	Total costs	Avg cost
SEPTICEMIA AND DISSEMINATED INFECTIONS	11830	220,967,975.77	18,678.61
INFECTIOUS AND PARASITIC DISEASES INCLUDING HIV WITH O.R. PROCEDURE	1622	67,194,738.04	41,427.09
HEART FAILURE	4566	63,671,495.57	13,944.70
VAGINAL DELIVERY	7240	53,074,031.29	7,330.67
PERCUTANEOUS STRUCTURAL CARDIAC PROCEDURES	811	45,043,964.80	55,541.26
RESPIRATORY FAILURE	2585	37,102,564.13	14,353.02
CESAREAN SECTION WITHOUT STERILIZATION	3397	37,079,049.11	10,915.23
Total	42175	765,941,003.16	18,161.02

FULL DRG CUMULATIVE COST CURVE (324 DRGS)



Facility name

All

Sub region

All

Severity

All

2.95

Charge to Cost Ratio

4.78

Highest Facility Ratio

1.03

Lowest Facility Ratio

3.10

National Avg Charge ...

3.40

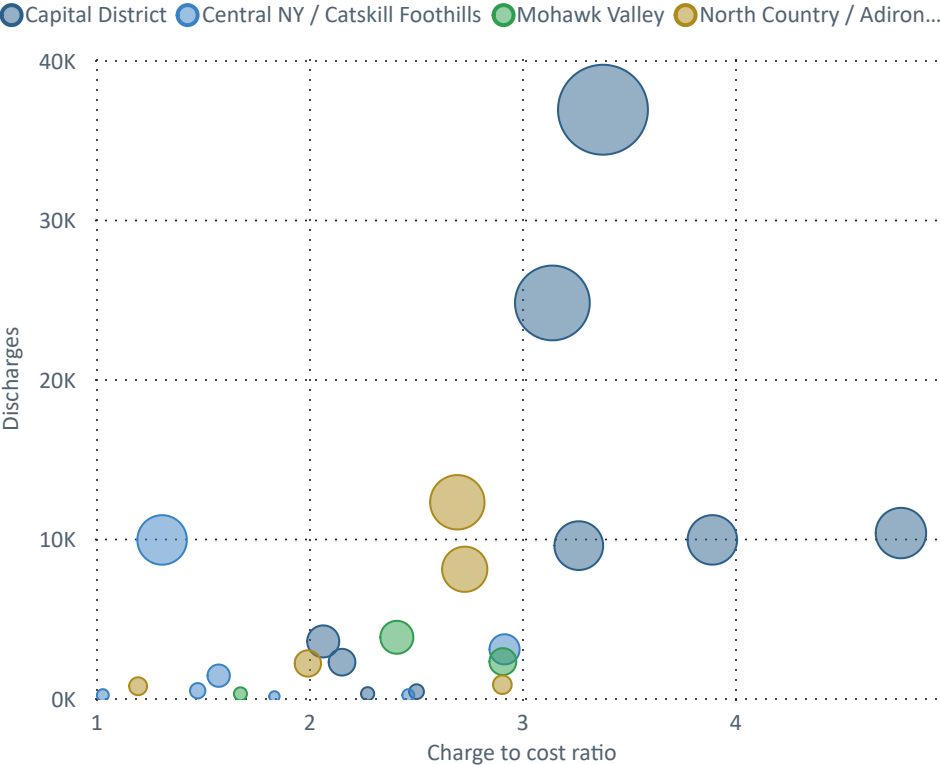
National Avg Charge ...

Charge-to-Cost Ratio (SPARCS derived-cost basis)

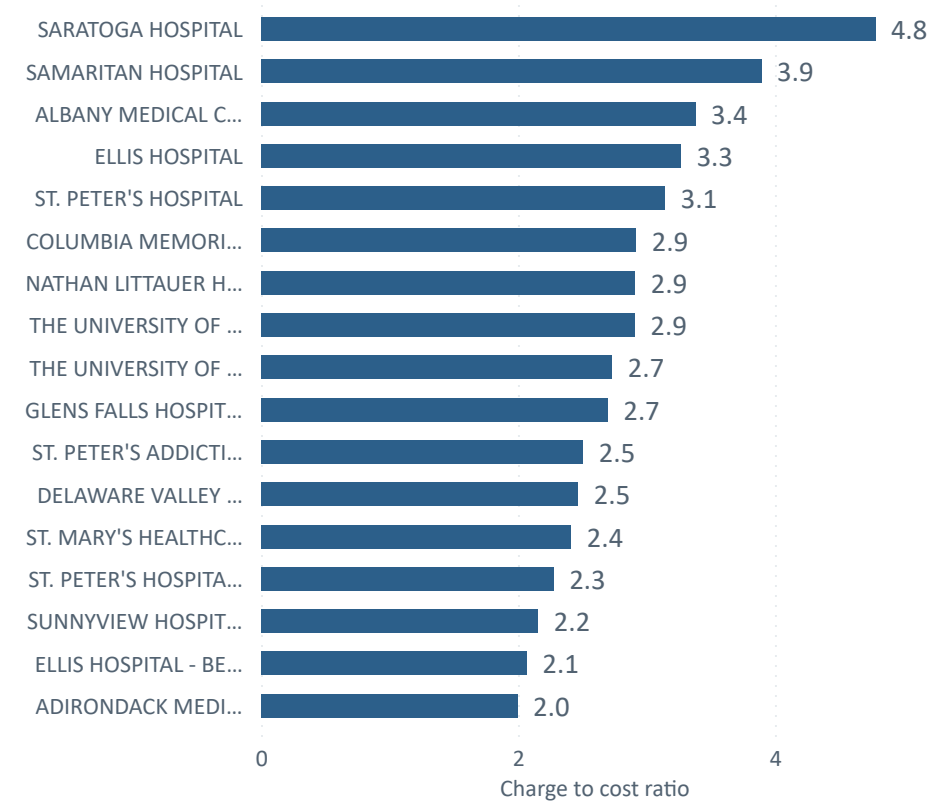
Ratio of billed charges to SPARCS estimated costs. SPARCS costs are derived from cost-to-charge ratios, so this reflects each facility's assigned CCR, not independently observed pricing.

SPARCS total_costs is estimated by applying cost-to-charge ratios to charges, not measured from hospital accounting. Within a facility, ~92% of cost variation is explained by charges, so this ratio largely restates each facility's assigned CCR. Read cross-facility differences as directional, and compare to the national benchmark rather than treating any single facility's ratio as a pricing decision.

CHARGE TO COST RATIO, DISCHARGES AND LOW-VOLUME FLAG BY FACILITY



CHARGE TO COST RATIO BY FACILITY, SORTED DESCENDING



Facility name

All

Sub region

All

Severity

All

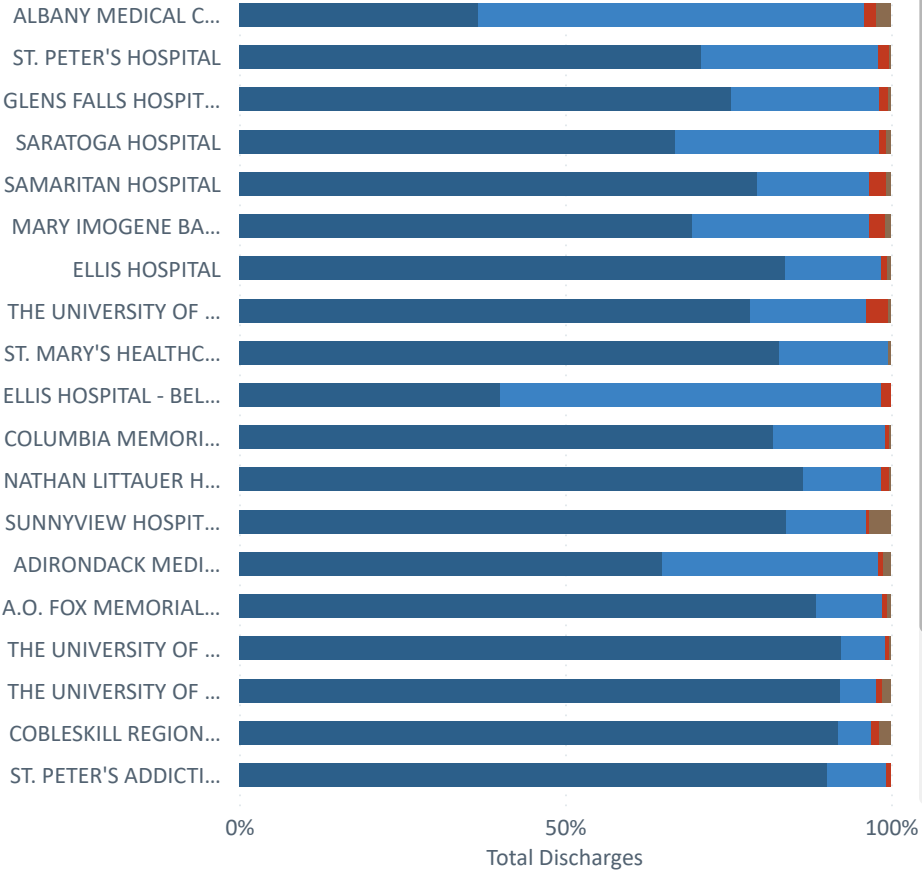
"Managed Care, Unspecified" (10.7% of discharges) may include Medicaid managed care; the band shows the range.

64.83%

Government Share

DISCHARGES AND GOVERNMENT SHARE BY FACILITY AND PAYER CATEGORY (TOP 12 SHOWN)

Government Commercial Self-Pay Other

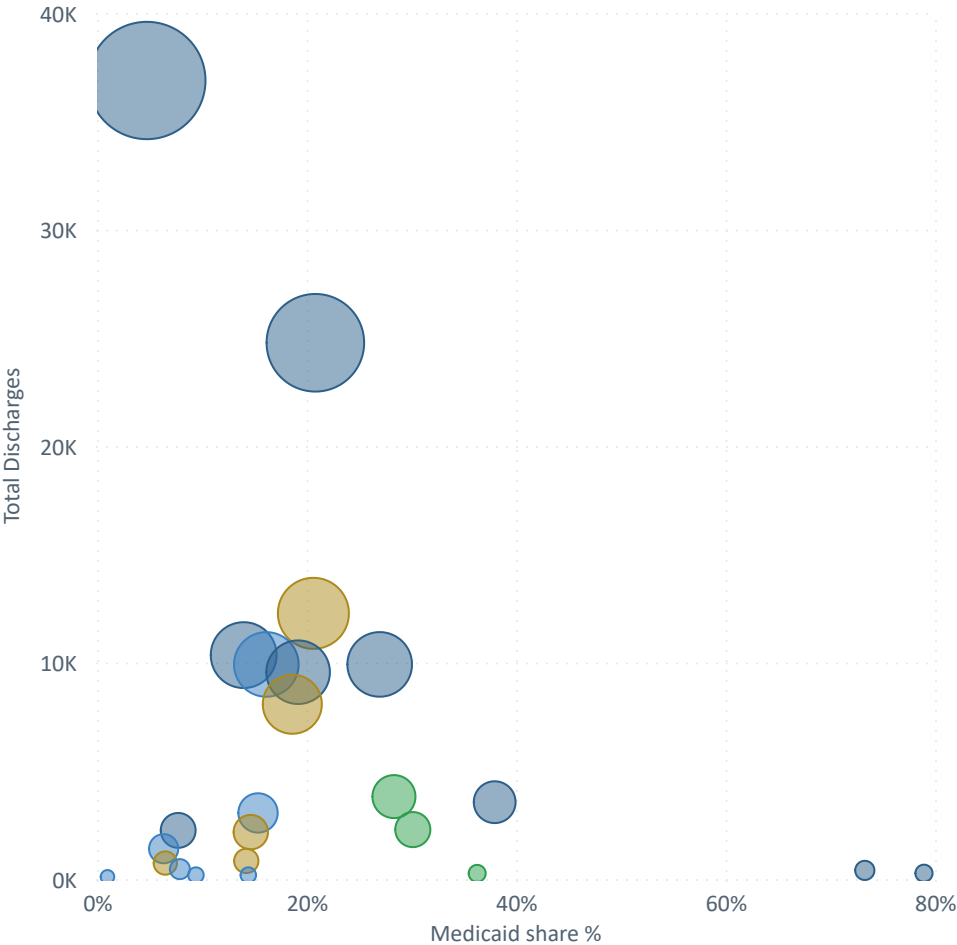


16.38%

Medicaid Share (reported floor)

MEDICAID SHARE AND DISCHARGES BY FACILITY AND SUB REGION

Capital District Central NY / Catskill Foothills Mohawk Valley North Country / Adiron...



Facility name

All

Sub region

All

Severity

All

5.45

Avg Length of Stay (Days)

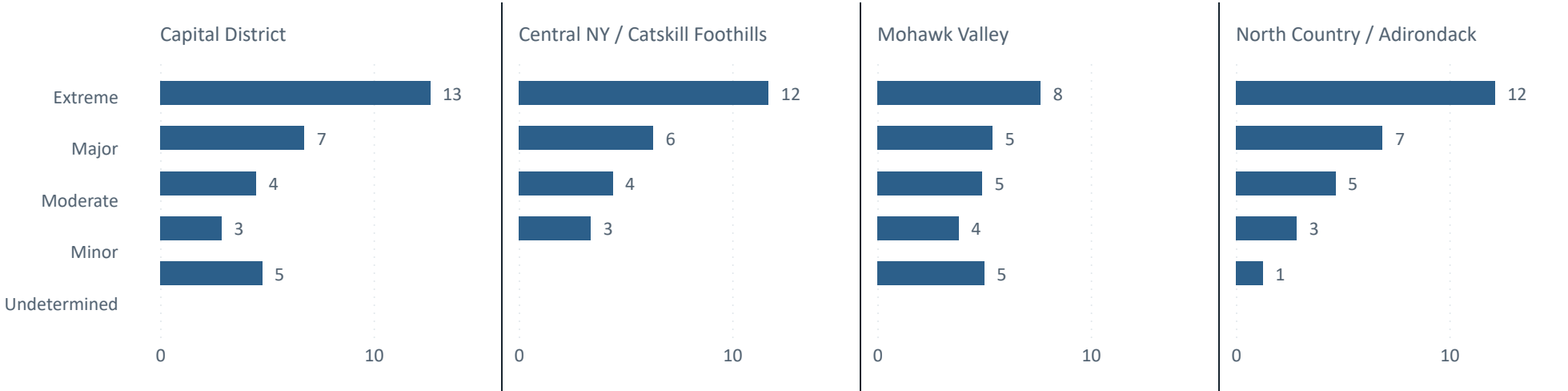
3.00

Median Length of Stay (Days)

0.04%

Pct Censored Stays

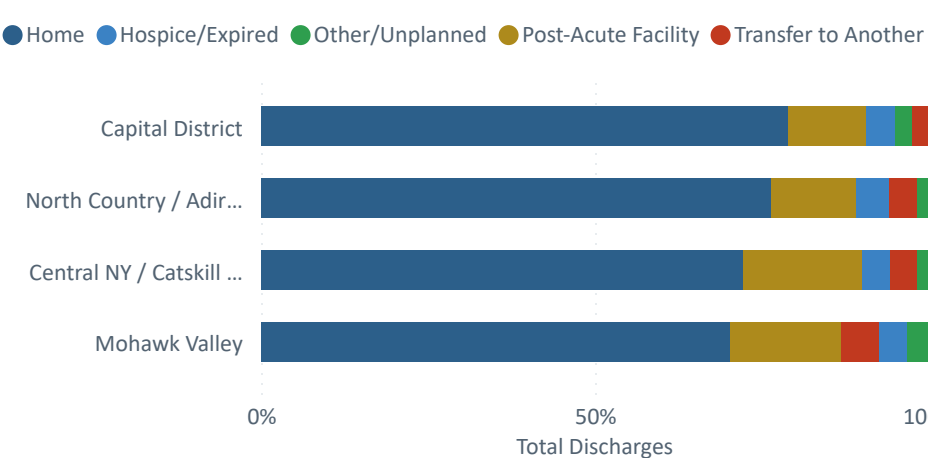
AVG LENGTH OF STAY (DAYS) BY SEVERITY AND SUB REGION



LENGTH OF STAY BY FACILITY AND SEVERITY (ILLUSTRATIVE ROWS)

Facility	Discharges	Avg LOS	Median LOS	% Censored
ALBANY MEDICAL CENTER HOSPITAL	36882	6.34	4.00	0.05%
ST. PETER'S HOSPITAL	24768	4.37	3.00	
GLENS FALLS HOSPITAL	12271	4.61	3.00	0.02%
SARATOGA HOSPITAL	10341	4.54	3.00	0.02%
SAMARITAN HOSPITAL	9912	4.72	3.00	0.04%
MARY IMOGENE BASSETT HOSPITAL	9910	4.80	3.00	0.01%
ELLIS HOSPITAL	9552	5.81	4.00	0.05%
THE UNIVERSITY OF VERMONT HEALTH NETWORK - CHAMPLAIN VALLEY PHYSICIANS HOSPITAL	8072	6.26	3.00	0.17%
ST. MARY'S HEALTHCARE	3806	4.70	4.00	0.02%
Total	143613	5.45	3.00	0.04%

DISCHARGES BY SUB REGION AND PATIENT DISPOSITION GROUP



Facility name

All

Sub region

All

Severity

All

61.85%

ED Utilization Pct

37.22%

High Severity Pct

REGIONAL UTILIZATION SCORECARD

Sub region	Discharges	% of Volume	ED Utilization %	High Severity %	Avg LOS	Avg Cost/Discharge	Discharges per 10k pop
Capital District	97911	68.18%	59.40%	38.46%	5.56	15,434.06	1,080.70
North Country / Adirondack	24080	16.77%	69.23%	33.18%	5.26	15,281.11	1,058.69
Central NY / Catskill Foothills	15275	10.64%	61.86%	37.45%	5.28	20,616.93	783.68
Mohawk Valley	6347	4.42%	71.50%	32.85%	4.90	11,307.33	623.96
Total	143613	100.00%	61.85%	37.22%	5.45	15,777.29	1,004.23

Mohawk Valley: lowest per-capita utilization, highest ED share

Mohawk Valley shows the lowest discharges-per-resident and the highest ED share. Because this counts discharges where the hospital sits, not where the patient lives, this pattern is consistent with residents leaving the region for planned care and/or constrained non-emergency access - it is a signal to investigate, not a settled utilization rate.

Per-capita = discharges at in-region hospitals / in-region residents; the two populations are not identical (patients travel). Directional only.